Department of Health Care Finance

www.dhcf.dc.gov
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Table HT0-1

Description	FY 2023 Actual	FY 2024 Actual	FY 2025 Approved	FY 2026 Approved	% Change from FY 2025
OPERATING BUDGET	$4,442,557,965	$4,401,503,659	$4,867,809,070	$5,504,769,104	13.1
FTEs	299.1	331.8	378.1	390.9	3.4
CAPITAL BUDGET	$126,041,063	$147,131,622	$0	$0	N/A
FTEs	0.0	0.0	0.0	0.0	N/A

The mission of the Department of Health Care Finance (DHCF) is to improve health outcomes by providing access to comprehensive, cost-effective, and quality health care services for residents of the District of Columbia.

Summary of Services

The Department of Health Care Finance provides health care services to low-income children, adults, the elderly, and persons with disabilities. More than 315,000 District of Columbia residents (approximately 45 percent of all residents) receive health care services through DHCF's Medicaid and Alliance programs. DHCF strives to provide these services in the most appropriate and cost-effective settings possible.

The agency's FY 2026 approved budget is presented in the following tables:

FY 2026 Approved Gross Funds Operating Budget and FTEs, by Revenue Type

Table HT0-2 contains the approved FY 2026 budget and approved Full-Time Equivalents by revenue type compared to the FY 2025 approved budget. It also provides FY 2023 and FY 2024 actual data.

Table HT0-2
(dollars in thousands)

	Dollars in Thousands						Full-Time Equivalents					
Appropriated Fund	Actual FY 2023	Actual FY 2024	Approved FY 2025	Approved FY 2026	Change from FY 2025	% Change*	Actual FY 2023	Actual FY 2024	Approved FY 2025	Approved FY 2026	Change from FY 2025	% Change
GENERAL FUND												
Local Funds	949,851	1,049,998	1,043,110	1,054,707	11,597	1.1	151.2	146.6	172.4	176.0	3.5	2.1
Dedicated Taxes	96,621	114,647	240,410	285,940	45,531	18.9	5.2	7.6	7.2	6.6	-0.6	-7.7

Table HT0-2

(dollars in thousands)

	Dollars in Thousands						Full-Time Equivalents					
Appropriated Fund	Actual FY 2023	Actual FY 2024	Approved FY 2025	Approved FY 2026	Change from FY 2025	% Change*	Actual FY 2023	Actual FY 2024	Approved FY 2025	Approved FY 2026	Change from FY 2025	% Change
Special Purpose Revenue Funds	2,577	3,928	7,008	5,492	-1,516	-21.6	13.8	14.0	15.6	15.0	-0.6	-3.5
TOTAL FOR GENERAL FUND	**1,049,049**	**1,168,573**	**1,290,528**	**1,346,139**	**55,611**	**4.3**	**170.2**	**168.3**	**195.2**	**197.6**	**2.4**	**1.3**
FEDERAL RESOURCES												
Federal Grant Funds	4,007	2,218	5,136	10,400	5,264	102.5	0.0	0.0	5.0	8.0	3.0	60.0
Federal Medicaid Payments	3,389,502	3,230,713	3,572,145	4,148,230	576,085	16.1	128.9	163.5	177.9	185.3	7.3	4.1
TOTAL FOR FEDERAL RESOURCES	**3,393,509**	**3,232,931**	**3,577,281**	**4,158,630**	**581,349**	**16.3**	**128.9**	**163.5**	**182.9**	**193.3**	**10.3**	**5.7**
GROSS FUNDS	**4,442,558**	**4,401,504**	**4,867,809**	**5,504,769**	**636,960**	**13.1**	**299.1**	**331.8**	**378.1**	**390.9**	**12.8**	**3.4**

*Percent change is based on whole dollars.

Note: If applicable, for a breakdown of each Grant (Federal and Private) and Special Purpose Revenue type, please refer to **Schedule 80 Agency Summary by Revenue Source** in the **FY 2026 Operating Appendices** located on the Office of the Chief Financial Officer's website.

In FY 2023, the Intra-District process that required duplicated budget in the agencies providing services (seller agencies) was eliminated and replaced by interagency projects. For more detailed information regarding the approved funding for interagency projects funded within this agency, please see **Appendix H, FY 2026 Interagency Budgets**, in the **Executive Summary, Volume 1.**

FY 2026 Approved Operating Budget, by Account Group

Table HT0-3 contains the approved FY 2026 budget at the Account Group level compared to the FY 2025 approved budget. It also provides FY 2023 and FY 2024 actual expenditures.

Table HT0-3

(dollars in thousands)

Account Group	Actual FY 2023	Actual FY 2024	Approved FY 2025	Approved FY 2026	Change from FY 2025	Percentage Change*
701100C - Continuing Full Time	30,652	34,338	39,864	41,181	1,317	3.3
701200C - Continuing Full Time - Others	1,438	1,658	1,847	2,550	703	38.1
701300C - Additional Gross Pay	324	323	0	2	2	N/A
701400C - Fringe Benefits - Current Personnel	7,361	8,118	9,426	10,101	675	7.2
701500C - Overtime Pay	29	124	0	0	0	N/A
SUBTOTAL PERSONNEL SERVICES (PS)	**39,804**	**44,560**	**51,137**	**53,834**	**2,697**	**5.3**
711100C - Supplies and Materials	14,286	110	176	134	-42	-23.8
712100C - Energy, Communications and Building Rentals	2,400	2,302	2,578	2,638	60	2.3
713100C - Other Services and Charges	762	3,194	4,336	4,200	-136	-3.1
713101C - Security Services	104	43	123	140	17	14.3
713200C - Contractual Services - Other	130,364	145,909	185,005	192,083	7,077	3.8
714100C - Government Subsidies and Grants	4,248,607	4,195,643	4,610,810	5,238,838	628,028	13.6
715100C - Other Expenses	13	0	0	0	0	N/A
715200C - P-Card Clearing Account Budget Tracking	1	0	0	0	0	N/A
717100C - Purchases Equipment and Machinery	6,218	9,741	13,644	12,895	-749	-5.5
717200C - Rentals Equipment and Other	0	0	0	7	7	N/A
SUBTOTAL NONPERSONNEL SERVICES (NPS)	**4,402,754**	**4,356,943**	**4,816,672**	**5,450,935**	**634,263**	**13.2**
GROSS FUNDS	**4,442,558**	**4,401,504**	**4,867,809**	**5,504,769**	**636,960**	**13.1**

*Percent change is based on whole dollars.

FY 2026 Approved Operating Budget and FTEs, by Division/Program and Activity

Table HT0-4 contains the approved FY 2026 budget by division/program and activity compared to the FY 2025 approved budget. It also provides FY 2023 and FY 2024 actual data. For a more comprehensive explanation of divisions/programs and activities, please see the Division/Program Description section, which follows the table.

Table HT0-4

(dollars in thousands)

	Dollars in Thousands					Full-Time Equivalents				
Division/Program and Activity	Actual FY 2023	Actual FY 2024	Approved FY 2025	Approved FY 2026	Change from FY 2025	Actual FY 2023	Actual FY 2024	Approved FY 2025	Approved FY 2026	Change from FY 2025
(AFO000) AGENCY FINANCIAL OPERATIONS										
(AFO002) Agency Accounting Services	3,353	3,927	7,249	6,463	-786	7.2	8.1	10.0	11.0	1.0
(AFO003) Agency Budgeting and Financial Management Services	589	751	836	889	53	4.8	5.4	6.0	7.0	1.0
(AFO005) Agency /Cluster Financial Executive Administration Services	326	354	369	374	5	1.6	1.8	2.0	2.0	0.0
(AFO011) P-Card Clearing	1	5	0	0	0	0.0	0.0	0.0	0.0	0.0
SUBTOTAL (AFO000) AGENCY FINANCIAL OPERATIONS	**4,270**	**5,036**	**8,454**	**7,726**	**-727**	**13.6**	**15.3**	**18.0**	**20.0**	**2.0**
(AMP000) AGENCY MANAGEMENT PROGRAM										
(AMP003) Communications	649	817	36	36	0	4.0	4.5	0.0	0.0	0.0
(AMP005) Contracting and Procurement	1,960	1,576	2,076	1,869	-207	12.0	11.7	14.0	14.0	0.0
(AMP006) Customer Experience	3,144	2,846	0	0	0	20.4	20.9	0.0	0.0	0.0
(AMP010) Grants Administration	10,824	1,692	0	0	0	2.3	3.7	0.0	0.0	0.0
(AMP011) Human Resource Services	1,044	982	1,514	1,244	-271	9.6	9.0	8.0	8.0	0.0
(AMP012) Information Technology Services	5,538	5,620	12,240	13,787	1,547	5.1	7.0	17.9	17.0	-0.9
(AMP014) Legal Services	1,220	1,221	1,471	1,480	9	7.9	8.2	8.8	8.9	0.0
(AMP019) Property, Asset, and Logistics Management	2,751	6,083	3,978	3,793	-186	4.8	14.6	6.0	7.0	1.0
(AMP021) Rates, Reimbursement, Financial Analysis	3,407	4,225	0	0	0	12.7	16.3	0.0	0.0	0.0
(AMP026) Training and Development	1,040	0	0	0	0	0.0	0.0	0.0	0.0	0.0
(AMP030) Executive Administration	1,226	972	1,445	1,691	245	1.6	6.3	9.0	10.0	1.0
(AMP037) Senior Deputy Director/Medicaid Director	0	0	4,108	14,146	10,038	0.0	0.0	14.0	15.0	1.0
(AMP038) Senior Deputy Director/Finance	0	0	6,395	6,602	207	0.0	0.0	20.0	23.0	3.0
(AMP039) Chief Operating Office	0	0	2,494	1,468	-1,026	0.0	0.0	8.0	7.0	-1.0
(AMP040) Data Analytics and Research Administration	0	0	2,015	2,523	509	0.0	0.0	11.0	13.0	2.0
(AMP041) Program Integrity	0	0	4,754	5,565	810	0.0	0.0	33.0	36.0	3.0
(AMP042) Health Care Ombudsman	0	0	4,957	5,342	385	0.0	0.0	24.0	26.0	2.0
SUBTOTAL (AMP000) AGENCY MANAGEMENT PROGRAM	**32,803**	**26,033**	**47,485**	**59,546**	**12,061**	**80.5**	**102.2**	**173.7**	**184.9**	**11.1**

Table HT0-4

(dollars in thousands)

Division/Program and Activity	Dollars in Thousands					Full-Time Equivalents				
	Actual FY 2023	Actual FY 2024	Approved FY 2025	Approved FY 2026	Change from FY 2025	Actual FY 2023	Actual FY 2024	Approved FY 2025	Approved FY 2026	Change from FY 2025
(AMP066) HEALTH CARE DELIVERY MANAGEMENT										
(AMP043) Health Care Delivery Management Support Services	0	0	1,443	1,509	65	0.0	0.0	9.0	10.0	1.0
(AMP044) Managed Care Management	0	0	9,585	9,425	-160	0.0	0.0	7.0	6.0	-1.0
(AMP045) Children's Health Services	0	0	1,292	1,237	-55	0.0	0.0	4.0	4.0	0.0
(AMP046) Health Care Quality and Health Outcomes	0	0	2,948	2,635	-313	0.0	0.0	7.0	7.0	0.0
(AMP047) Clinicians, Rx and Acute Care	0	0	9,566	0	-9,566	0.0	0.0	0.0	0.0	0.0
SUBTOTAL (AMP066) HEALTH CARE DELIVERY MANAGEMENT	**0**	**0**	**24,835**	**14,806**	**-10,029**	**0.0**	**0.0**	**27.0**	**27.0**	**0.0**
(AMP067) LONG TERM CARE PROGRAM										
(AMP048) Long Term Care Support Services	0	0	859	809	-50	0.0	0.0	6.0	6.0	0.0
(AMP049) Oversight	0	0	2,100	2,271	171	0.0	0.0	17.0	17.0	0.0
(AMP050) Operations	0	0	12,358	18,654	6,296	0.0	0.0	16.6	16.0	-0.6
(AMP051) Intake and Assessment	0	0	9,841	8,442	-1,399	0.0	0.0	6.4	7.0	0.6
SUBTOTAL (AMP067) LONG TERM CARE PROGRAM	**0**	**0**	**25,158**	**30,176**	**5,017**	**0.0**	**0.0**	**46.0**	**46.0**	**0.0**
(AMP068) HEALTH CARE POLICY AND RESEARCH										
(AMP052) Health Care Policy	0	0	1,004	1,021	17	0.0	0.0	7.0	7.0	0.0
(AMP053) Health Care Policy and Research Support Services	0	0	1,605	1,197	-407	0.0	0.0	3.0	4.0	1.0
(AMP054) Eligibility Policy & Oversight	0	0	3,843	4,402	559	0.0	0.0	29.0	29.0	0.0
SUBTOTAL (AMP068) HEALTH CARE POLICY AND RESEARCH	**0**	**0**	**6,452**	**6,621**	**169**	**0.0**	**0.0**	**39.0**	**40.0**	**1.0**
(AMP069) DC ACCESS SYSTEM										
(AMP055) DCAS Program Management	0	0	1,807	1,828	21	0.0	0.0	4.3	4.3	0.0
(AMP056) DCAS Project Management	0	0	4,328	3,923	-405	0.0	0.0	6.9	5.2	-1.7
(AMP057) DCAS HHS Functional	0	0	814	851	37	0.0	0.0	6.0	6.1	0.0
(AMP058) DCAS Organizational Change Management	0	0	13,203	12,422	-781	0.0	0.0	6.9	6.9	0.0
(AMP059) DCAS Information Technology	0	0	57,268	53,078	-4,190	0.0	0.0	6.9	6.9	0.0
SUBTOTAL (AMP069) DC ACCESS SYSTEM	**0**	**0**	**77,420**	**72,102**	**-5,318**	**0.0**	**0.0**	**31.1**	**29.4**	**-1.7**
(AMP070) HEALTH CARE OPERATIONS										
(AMP060) Claims Management	0	0	59,722	64,208	4,486	0.0	0.0	14.0	14.0	0.0
(AMP061) HCOA Support Services	0	0	454	483	29	0.0	0.0	3.0	3.0	0.0
(AMP062) Public and Private Provider Services	0	0	3,665	6,425	2,760	0.0	0.0	9.0	8.0	-1.0
SUBTOTAL (AMP070) HEALTH CARE OPERATIONS	**0**	**0**	**63,841**	**71,116**	**7,275**	**0.0**	**0.0**	**26.0**	**25.0**	**-1.0**

Table HT0-4

(dollars in thousands)

	Dollars in Thousands					Full-Time Equivalents				
Division/Program and Activity	Actual FY 2023	Actual FY 2024	Approved FY 2025	Approved FY 2026	Change from FY 2025	Actual FY 2023	Actual FY 2024	Approved FY 2025	Approved FY 2026	Change from FY 2025
(AMP071) HEALTH CARE REFORM AND INNOVATION										
(AMP063) HC Reform and Innovative Support Services	0	0	454	611	157	0.0	0.0	3.0	4.0	1.0
(AMP064) Grants Administration Function	0	0	6,252	10,208	3,956	0.0	0.0	3.0	4.0	1.0
(AMP065) HIE: Health Information Exchange	0	0	11,321	11,221	-100	0.0	0.0	7.0	7.0	0.0
SUBTOTAL (AMP071) HEALTH CARE REFORM AND INNOVATION	**0**	**0**	**18,027**	**22,040**	**4,013**	**0.0**	**0.0**	**13.0**	**15.0**	**2.0**
(HS0023) ELIGIBILITY & ENROLLMENT										
(H02301) E & E Systems	40,471	46,339	99	0	-99	21.8	15.6	1.1	0.0	-1.1
(H02302) E&E Oversight & Management	787	812	0	0	0	13.6	9.0	0.0	0.0	0.0
(H02305) E&E Support - PMO/SME - DCAS	14,767	21,832	0	0	0	32.6	15.1	0.0	0.0	0.0
SUBTOTAL (HS0023) ELIGIBILITY & ENROLLMENT	**56,025**	**68,983**	**99**	**0**	**-99**	**68.1**	**39.7**	**1.1**	**0.0**	**-1.1**
(HS0024) HEALTH CARE ANALYTICS										
(H02401) Health Care Agency Monitoring and Kpis	1,200	1,594	0	0	0	7.2	8.1	0.0	0.0	0.0
(H02402) Health Care Data Analytics	1,164	1,369	0	0	0	10.4	8.1	0.0	0.0	0.0
(H02403) Health Care Innovation	9,551	7,682	0	0	0	6.8	7.4	0.0	0.0	0.0
SUBTOTAL (HS0024) HEALTH CARE ANALYTICS	**11,915**	**10,646**	**0**	**0**	**0**	**24.4**	**23.6**	**0.0**	**0.0**	**0.0**
(HS0025) MMIS SYSTEM AND INFRASTRUCTURE										
(H02501) Claims Processing & Quality Assurance/Control	43,855	47,021	0	0	0	13.5	15.4	0.0	0.0	0.0
SUBTOTAL (HS0025) MMIS SYSTEM AND INFRASTRUCTURE	**43,855**	**47,021**	**0**	**0**	**0**	**13.5**	**15.4**	**0.0**	**0.0**	**0.0**
(HS0026) PROGRAM OVERSIGHT										
(H02601) Assessments and Care Coordination	972	1,009	0	0	0	6.3	7.2	0.0	0.0	0.0
(H02602) Fraud, Waste, and Abuse	3,495	4,155	0	0	0	23.0	25.3	0.0	0.0	0.0
(H02603) Policy	2,784	4,757	0	0	0	7.2	28.0	0.0	0.0	0.0
(H02604) Provider Oversight	38,841	40,647	2,125	0	-2,125	51.8	56.9	0.0	0.0	0.0
(H02605) Quality & Health Outcomes	3,163	3,292	501	395	-106	9.3	11.8	3.2	3.6	0.4
SUBTOTAL (HS0026) PROGRAM OVERSIGHT	**49,256**	**53,859**	**2,626**	**395**	**-2,231**	**97.6**	**129.2**	**3.2**	**3.6**	**0.4**

Table HT0-4

(dollars in thousands)

	Dollars in Thousands					Full-Time Equivalents				
Division/Program and Activity	Actual FY 2023	Actual FY 2024	Approved FY 2025	Approved FY 2026	Change from FY 2025	Actual FY 2023	Actual FY 2024	Approved FY 2025	Approved FY 2026	Change from FY 2025
(HS0027) PROVIDER SERVICES										
(H02703) 1115/1915 - Medicaid	456,082	413,218	439,257	498,998	59,741	0.0	0.0	0.0	0.0	0.0
(H02704) FFS - Childless Adults (Group 8)	8,829	9,733	49,225	69,896	20,670	0.0	0.0	0.0	0.0	0.0
(H02705) FFS - Chip	7,001	8,368	8,566	13,375	4,809	0.0	0.0	0.0	0.0	0.0
(H02706) FFS - Medicaid	1,345,709	1,449,063	1,223,326	1,433,980	210,654	1.4	1.7	0.0	0.0	0.0
(H02707) MCO - Alliance	111,194	147,448	132,644	0	-132,644	0.0	0.0	0.0	0.0	0.0
(H02708) MCO - Childless Adults (Group 8)	770,922	712,779	931,882	1,129,395	197,513	0.0	0.0	0.0	0.0	0.0
(H02709) MCO - Chip	52,094	54,132	73,587	97,779	24,192	0.0	0.0	0.0	0.0	0.0
(H02710) MCO - Immigrant Children	15,119	19,114	18,284	0	-18,284	0.0	0.0	0.0	0.0	0.0
(H02711) MCO - Medicaid	1,248,649	1,130,795	1,529,116	1,658,576	129,461	0.0	0.0	0.0	0.0	0.0
(H02712) MCO- Waiver	146,011	151,402	187,301	196,846	9,546	0.0	0.0	0.0	0.0	0.0
(H02713) HCBS ARPA Initiative	82,825	78,804	225	0	-225	0.0	4.6	0.0	0.0	0.0
(H02714) Indigent Care	0	0	0	121,396	121,396	0.0	0.0	0.0	0.0	0.0
SUBTOTAL (HS0027) PROVIDER SERVICES	**4,244,434**	**4,174,857**	**4,593,413**	**5,220,241**	**626,829**	**1.4**	**6.3**	**0.0**	**0.0**	**0.0**
(PRG000) NO PROGRAM										
(PRG001) No Program	0	15,069	0	0	0	0.0	0.0	0.0	0.0	0.0
SUBTOTAL (PRG000) NO PROGRAM	**0**	**15,069**	**0**	**0**	**0**	**0.0**	**0.0**	**0.0**	**0.0**	**0.0**
TOTAL APPROVED OPERATING BUDGET	**4,442,558**	**4,401,504**	**4,867,809**	**5,504,769**	**636,960**	**299.1**	**331.7**	**378.1**	**390.9**	**12.8**

(Change is calculated by whole numbers and numbers may not add up due to rounding)

Note: For more detailed information regarding the approved funding for the activities within this agency's programs, please see **Schedule 30-PBB Program Summary by Activity.** For detailed information on this agency's Cost Center structure as reflected in the District's Chart of Accounts, please see **Schedule 30-CC FY 2026 Approved Operating Budget and FTEs, by Division/Office.** The schedules can be found in the **FY 2026 Operating Appendices** located on the Office of the Chief Financial Officer's website. Additional information on this agency's interagency agreements can be found in **Appendix H** in the **Executive Summary, Volume 1.**

Division Description

The Department of Health Care Finance operates through the following 10 divisions:

Health Care Delivery Management – ensures that quality services and practices pervade all activities that affect the delivery of health care to beneficiaries served by the District's Medicaid, Children's Health Insurance Program (CHIP), and Alliance programs. HCDM accomplishes this through informed benefit design; use of prospective, concurrent and retrospective utilization management; ongoing program evaluation; and the application of continuous quality measurement and improvement practices in furnishing preventive, acute, and chronic/long-term care services to children and adults through DHCF's managed care contractors and institutional and ambulatory fee-for-service providers.

This division contains the following 4 activities:

- **Health Care Delivery Management Support Services** – provides administrative support functions to the Health Care Delivery Management division;

- **Managed Care Management** – provides oversight, evaluation, and enforcement of contracts with organizations managing the care and service delivery of Medicaid and Alliance beneficiaries, along with providing oversight and enrollment of eligible beneficiaries;
- **Children's Health Services** – develops, implements, and monitors policies, benefits and practices for children's health care services, including HealthCheck/EPSDT, CHIP, and the Immigrant Children's Program; and
- **Health Care Quality and Health Outcomes** – continuously improves the quality (safe, effective, patient-centered, timely, efficient, and equitable services) of health care delivered by programs administered by DHCF; and ensures that quality and performance improvement principles and practices pervade all the components and activities that impact the delivery and outcomes of health care services to patients served by the District's Medicaid, CHIP, and Alliance programs.

Long-Term Care Program – provides operations support, oversight and monitoring of programs targeted to the elderly, persons with physical disabilities, persons with intellectual and developmental disabilities and individuals who are dually eligible for both Medicare and Medicaid. Through program development, day-to-day operations, and oversight and monitoring activities, the LTCA also ensures access to needed cost-effective, high-quality extended and long-term care services for Medicaid beneficiaries residing in home and community-based or institutional settings. The office also provides contract management of various long-term care services and supports contracts, including the long-term care assessment contract, the contracts for the Dual Eligible Special Needs Plan (D-SNP) and Program for All-Inclusive Care for the Elderly (PACE) programs, and more.

This division contains the following 4 activities:

- **Long-Term Care Support Services** – provides administrative support functions to the Long-Term Care Administration;
- **Oversight** – provides quality oversight and assurance, including compliance with mandatory Centers for Medicare and Medicaid Services (CMS) assurances, oversight and audits/site visits, and management of corrective action plans;
- **Operations** – provides day-to-day operations support for LTC partners, providers and beneficiaries to ensure service delivery; training and technical assistance to providers; provider readiness; and grievance and appeals triage and resolution; and
- **Intake and Assessment** – clinical operations unit responsible for assessment and intake processes, coordination with the DC Department on Aging and Community Living (DACL), and other clinical processes within LTCA's purview.

Health Care Policy and Research – maintains the Medicaid and CHIP state plans that govern eligibility, scope of benefits, and reimbursement policies for the District's Medicaid and CHIP programs; develops policy for the Health Care Alliance program and other publicly funded health care programs that are administered or monitored by DHCF based on sound analysis of local and national health care and reimbursement policies and strategies; and ensures coordination and consistency among health care and reimbursement policies developed by the various divisions within DHCF.

This division contains the following 3 activities:

- **Health Care Policy** – maintains the Medicaid State Plan, which governs the eligibility, scope of benefits, and reimbursement policies of the Medicaid and CHIP programs;
- **Health Care Policy and Research Support Services** – provides administrative support to functions to the Health Care Policy and Planning Administration; and
- **Eligibility Policy and Oversight** – serves as liaison to District and federal agencies regarding eligibility-related matters; ensures collaboration and coordination between the agencies and facilitates compliance by the Department of Human Services' Economic Security Administration with DHCF eligibility policy; interprets federal and state eligibility rules and regulation; establishes eligibility policies and criteria for the Medicaid and CHIP programs, as well as the Health Care Alliance and the Immigrant Children's Program and determines eligibility for long term care services and supports and special eligibility programs; interprets and helps draft legislative changes, rules and regulations for the District regarding eligibility requirements; and manages the Optional State Supplement Payment Program for eligible District of Columbia residents residing in an adult foster care home.

DC Access System (DCAS) – has responsibility to design, develop, implement, and manage the DC Access System (DCAS), which is an integrated eligibility system for all health and human services for the District. In addition, this administration is responsible for supporting the functionality and funding for all components of DCAS and their seamless interface with the Health Benefits Exchange and Department of Human Services program components.

This division contains the following 5 activities:

- **DCAS Program Management** – manages all operational and functional activities related to the DCAS project;
- **DCAS Project Management** – manages all project management and functional activities related to the DCAS project;
- **DCAS HHS Functional** – joint oversight development execution of the DC access systems;
- **DCAS Organizational Change Management** – manages all historical, current, and forecasted project initiatives associated with Organization Change Management; and
- **DCAS Information Technology** – manages the operational tasks and maintenance for the DCAS project.

Health Care Operations – ensures the division of programs that pertain to the payment of claims and manages the fiscal agent contract, the administrative contracts, systems, and provider enrollment and requirements. The office provides contract management of the Pharmacy Benefits Manager, the Quality Improvement Organization contract, and the Medicaid Management Information System (MMIS) Fiscal Intermediary contract as well as additional administrative contracts.

This division contains the following 3 activities:

- **Claims Management** – oversees MMIS operations; systems requests; member services;
- **HCOA Support Services** – provides administrative support functions to the Health Care Operations divisions; and
- **Public and Private Provider Services** – manages the Administrative Services Organization contract, provider enrollment and recruitment, and internal and external provider services and inquiries. The office also maintains positive ongoing coordination and continuity with all public provider agencies of the District of Columbia government to enhance each agency's understanding of Medicaid reimbursement policies; is the accountable office within DHCF for implementation of policy that directly impacts other District agencies that serve as Medicaid providers; identifies opportunities to improve the reimbursement procedures of each agency; and works closely with these agencies to review federal policy to ensure that federal reimbursement is being maximized and compliance assured through claims processing and through program development.

Health Care Reform and Innovation – identifies, validates, and disseminates information about new health care models and payment approaches serving Medicaid beneficiaries with the goal of enhancing health care quality, improving care and outcomes, promoting health equity, and enhancing the value and efficiency of DHCF programs. The division creates and tests new delivery system and payment models among providers in the District and builds collaborative learning networks to facilitate innovation, implement effective practices, and facilitate technology improvements to support delivery system re-design and improvement.

This division contains the following 3 activities:

- **Health Care Reform and Innovative Support Services** – is responsible for advancing the use of information technology among health care providers in the District. These activities include HCRIA's responsibility to design, develop, implement, and sustain Health Information Exchange (HIE) activities. HIE's infrastructure provides the technology, processes, and operations needed to facilitate exchange of health information between health stakeholders. HCRIA's Health Information Technology (HIT) program offers incentives, outreach, and technical assistance to drive the adoption and use of Certified Electronic Health Records Technology by District health care providers. The program aligns with CMS's Meaningful Use framework;

- **Grant Administration Function** – develops and executes strategies for payment and delivery system reform in the District, including developing, implementing, and monitoring practice transformation activities as well as developing demonstration projects and grants to support various value–based purchasing and practice transformation strategies; and
- **Health Information Exchange** – responsible for advancing the use of digital health among health care providers in the District. This Division serves as the state health IT coordinator and regulates health information exchange (HIE). It is also responsible for the design, development, implementation, and sustainability of DC HIE connectivity and infrastructure, which enables operations needed to facilitate exchange of health information across the District's health system. Lastly, this Division offers incentives, outreach and technical assistance to drive adoption and meaningful use of digital health tools by District health care providers.

Program Oversight – to provide a reasonable and consistent oversight of the Medicaid program.

This division contains the following activity:

- **Quality and Health Outcomes** – ensures continuous improvements to the quality and performance principles and practices of the health care programs administered by DHCF.

Provider Services – provides payment to providers in the following: the Medicaid program, the Children's Health Insurance Program (CHIP) and the Alliance program.

This division contains the following 9 activities:

- **1115/1915 - Medicaid** – waiver programs offered through 1915 and 1115 authority providing care for home and community based services (1915) or demonstration programs(1115) to expand care beyond the traditional Medicaid program;
- **Fee for Services (FFS) - Childless Adults (Group 8)** – fee for Service provider payments for adults between the ages of 21 and 64 who do not have dependents;
- **FFS - Chip** – for Service provider payments for the Children's Health Insurance Program. This population is broken out due to a more favorable federal match percentage;
- **FFS Medicaid** – fee for services provider payments for the general Medicaid population;
- **MCO Childless Adults (Group 8)** – MCO payments for adults between the ages of 21 and 64 who do not have dependents;
- **MCO Chip** – MCO payments for the Children's Health Insurance Program. Provides coverage for children who are in families with income too high to qualify for Medicaid, but are too low to afford private coverage;
- **MCO Medicaid** – MCO payments for the general Medicaid population;
- **MCO Waiver** – dual Eligible Special Needs Program (DSNP), Medicare Advantage; and
- **Indigent Care** - program serving those uninsured and otherwise not eligible for Medicaid.

Agency Financial Operations – provides comprehensive and efficient financial management services to, and on behalf of, District agencies so that he financial integrity of the District of Columbia is maintained This division is standard for all agencies using performance/based budgeting.

Agency Management – provides for administrative support and the required tools to achieve operational and programmatic results. This division is standard for all agencies using performance-based budgeting.

The DHCF Agency Management structure also includes:

- **Office of the Senior Deputy Director/Medicaid Director (Office of the State Medicaid Director - OSMD)** – Responsible for overseeing the DC Medicaid Program, ensuring its affordability to the District of Columbia residents, including the office of the Chief Medical Officer;
- **Health Care Ombudsman's Office** – Responsible for assisting people in understanding their health care coverage and providing guidance, advocating, and assisting in the appeals process;
- **Office of the Senior Deputy Director, Finance (Office of Rates, Reimbursement and Financial Analysis - ORRFA)** – responsible for provider rate setting and reimbursement, program financial analysis of spending trends and utilization and actuarial oversight of the Managed Care Organizations;
- **Data Analytics and Research (DARA)** – Transforms data into meaningful information to support decision-making, effectiveness, and transparency in the District's Medicaid and locally funded health coverage programs; and
- **Program Integrity (PI)** – provide a reasonable and consistent system of oversight of the Medicaid program; reduce and eliminate fraud, waste, and abuse in the Medicaid Program.

Division Structure Change

The Department of Health Care Finance has no division structure changes in the FY 2026 approved budget.

FY 2025 Approved Budget to FY 2026 Approved Budget, by Revenue Type

Table HT0-5 itemizes the changes by revenue type between the FY 2025 approved budget and the FY 2026 approved budget. For a more comprehensive explanation of changes, please see the FY 2026 Approved Budget Changes section, which follows the table.

Table HT0-5

(dollars in thousands)

DESCRIPTION	DIVISION/PROGRAM	BUDGET	FTE
LOCAL FUNDS: FY 2025 Approved Budget and FTE		**1,043,110**	**172.4**
Removal of One-Time Funding	Multiple Programs	-100	0.0
LOCAL FUNDS: FY 2026 Recurring Budget		**1,043,010**	**172.4**
Increase: To support medical care costs	Provider Services	7,552	0.0
Increase: To adjust the Contractual Services budget	Multiple Programs	1,996	0.0
Increase: To align personnel services and Fringe Benefits with projected costs	Multiple Programs	843	2.4
Decrease: To realize savings in nonpersonnel services	Multiple Programs	-362	0.0
Decrease: Equipment cost savings	DC Access System	-6,223	0.0
Enhance: To support DCAS ($5.7m), equipment upgrades ($510k), MOU to support IVF treatment ($175k), and contractual services ($168k) (one-time)	Multiple Programs	6,531	0.0
Enhance: To support DSP wage enhancement (one-time)	Provider Services	6,510	0.0
Enhance: To support the net impact of lower Dedicated Tax revenue and Graduate Medical Education savings	Provider Services	3,374	0.0
Reduce: To reflect the proposed one-time reduction of step increases and associated fringe benefit costs	Multiple Programs	-138	0.0
Reduce: To support provider rate reductions and limited enrollment for people not otherwise eligible for Medicaid	Provider Services	-11,363	0.0

Table HT0-5

(dollars in thousands)

DESCRIPTION	DIVISION/PROGRAM	BUDGET	FTE
LOCAL FUNDS: FY 2026 Mayor's Proposed Budget		**1,051,730**	**174.9**
Enhance: To support the impact of lower inpatient revenue from revised ACR projections	Provider Services	2,297	0.0
Enhance: To support coverage extension for individuals in the Alliance program aged 21 to 25 (one-time)	Provider Services	1,801	0.0
Enhance: To provide durable medical equipment for adults	Provider Services	1,379	0.0
Enhance: To support extension period for new Alliance enrollees (one-time)	Provider Services	475	0.0
Enhance: To provide durable medical equipment for youth under 26 years old	Provider Services	345	0.0
Enhance: To support the Home Visiting/Nurse Family Partnership program	Health Care Reform and Innovation	300	0.0
Enhance: To support the continued use of all current enrollment certification methods for the Alliance program	Provider Services	198	0.0
Enhance: To support additional FTE(s)	Agency Management Program	95	1.1
Reduce: To realize programmatic cost savings in nonpersonnel services	Multiple Programs	-62	0.0
Reduce: To realize savings in contractual services	Multiple Programs	-522	0.0
Reduce: To align the budget with enrollment projections for the Alliance program	Provider Services	-3,329	0.0
LOCAL FUNDS: FY 2026 District's Approved Budget		**1,054,707**	**176.0**
DEDICATED TAXES: FY 2025 Approved Budget and FTE		**240,410**	**7.2**
Increase: To align the budget with projected revenues	Provider Services	62,023	0.0
Decrease: To align personnel services and Fringe Benefits with projected costs	Multiple Programs	-10	-0.6
Decrease: To align the Contractual Services budget with projected costs	Agency Financial Operations	-442	0.0
Reduce: To reflect the proposed one-time reduction of step increases and associated fringe benefit costs	Multiple Programs	-7	0.0
Reduce: To align the budget with projected revenue due to fewer beneficiaries enrolled in managed care services	Provider Services	-34,260	0.0
DEDICATED TAXES: FY 2026 Mayor's Proposed Budget		**267,714**	**6.6**
Enhance: To align budget with Outpatient Hospital Directed Payments Provider Fee revenue projection	Provider Services	38,337	0.0
Reduce: To align budget with Inpatient Hospital Directed Payments Provider Fee revenue projection	Provider Services	-20,111	0.0
DEDICATED TAXES: FY 2026 District's Approved Budget		**285,940**	**6.6**
SPECIAL PURPOSE REVENUE FUNDS: FY 2025 Approved Budget and FTE		**7,008**	**15.6**
Increase: To align personnel services and Fringe Benefits with projected costs	Multiple Programs	109	-0.6
Decrease: To align the budget with projected revenues	Multiple Programs	-1,625	0.0
Enhance: To support nonpersonnel services costs	Multiple Programs	15	0.0
Reduce: To reflect the proposed one-time reduction of step increases and associated fringe benefit costs	Multiple Programs	-15	0.0
SPECIAL PURPOSE REVENUE FUNDS: FY 2026 Mayor's Proposed Budget		**5,492**	**15.0**
No Change		0	0.0
SPECIAL PURPOSE REVENUE FUNDS: FY 2026 District's Approved Budget		**5,492**	**15.0**
FEDERAL GRANT FUNDS: FY 2025 Approved Budget and FTE		**5,136**	**5.0**
Increase: To align the budget with projected grant awards	Multiple Programs	4,805	0.0
Increase: To align personnel services and Fringe Benefits with projected costs	Multiple Programs	469	3.0
Reduce: To reflect the proposed one-time reduction of step increases and associated fringe benefit costs	Long Term Care Program	-10	0.0
FEDERAL GRANT FUNDS: FY 2026 Mayor's Proposed Budget		**10,400**	**8.0**
No Change		0	0.0
FEDERAL GRANT FUNDS: FY 2026 District's Approved Budget		**10,400**	**8.0**

Table HT0-5

(dollars in thousands)

DESCRIPTION	DIVISION/PROGRAM	BUDGET	FTE
FEDERAL MEDICAID PAYMENTS: FY 2025 Approved Budget and FTE		**3,572,145**	**177.9**
Increase: To adjust Federal match to align with Local budget	Multiple Programs	492,331	0.0
Increase: To adjust Federal match to align with Local personnel services budget	Multiple Programs	1,424	6.4
Enhance: To adjust for Medicaid reimbursements	Provider Services	14,901	0.0
Reduce: To reflect the proposed one-time reduction of step increases and associated fringe benefit costs	Multiple Programs	-141	0.0
FEDERAL MEDICAID PAYMENTS: FY 2026 Mayor's Proposed Budget		**4,080,660**	**184.4**
Enhance: To adjust for Medicaid reimbursements	Provider Services	67,493	0.0
Enhance: To support a partial FTE	Agency Management Program	78	0.9
FEDERAL MEDICAID PAYMENTS: FY 2026 District's Approved Budget		**4,148,230**	**185.2**
GROSS FOR HT0 - DEPARTMENT OF HEALTH CARE FINANCE		**5,504,769**	**390.9**

(Change is calculated by whole numbers and numbers may not add up due to rounding)

Note: For more detailed information regarding the approved funding for interagency projects funded within this agency, please see **Appendix H, FY 2026 Interagency Budgets, of the Executive Summary, Volume 1** located on the OCFO's website.

FY 2026 Approved Operating Budget Changes

Table HT0-6 contains the approved FY 2026 budget by fund compared to the FY 2025 approved budget.

Table HT0-6

Appropriated Fund	FY 2025 Approved	FY 2026 Approved	% Change from FY 2025
Local Funds	$1,043,110,088	$1,054,706,805	1.1
Dedicated Taxes	$240,409,641	$285,940,321	18.9
Special Purpose Revenue Funds	$7,008,261	$5,491,817	-21.6
Federal Grant Funds	$5,136,131	$10,399,776	102.5
Federal Medicaid Payments	$3,572,144,949	$4,148,230,385	16.1
GROSS FUNDS	**$4,867,809,070**	**$5,504,769,104**	**13.1**

Mayor's Proposed Budget

Increase: The Department of Health Care Finance's (DHCF) proposed Local funds budget submission includes a net increase of $7,552,401 in the Provider Services division. This adjustment is comprised of an increase of $43,797,527 to support the increasing cost of medical care for the most vulnerable beneficiaries, including a rate increase for dental services to ensure access and the implementation of CMS required coverage for youth; and is offset by savings of $36,245,126 to reflect reduced eligibility for participants with income levels above 138.0 percent of the federal poverty level; shifting their care to the Health Benefits Exchange by providing insurance through one of two benefits programs; adjusting programs serving those otherwise not eligible for Medicaid; and adjusting the impacts of provider rates by adjusting/delaying rebasing and/or eliminating inflation.

A proposed Local funds increase of $1,995,700 in nonpersonnel services across multiple divisions will support various program initiatives, primarily in contractual services costs. Additionally, DHCF proposes a Local funds increase of $842,937 and 2.4 Full-Time Equivalents (FTEs) across multiple divisions that will align the personnel services budget with projected salary and Fringe Benefit costs.

In Dedicated Tax funds, a proposed increase of $62,023,014 in subsidies to the Provider Services division is comprised of projected revenue increases of $25,089,799 in the Inpatient Directed Payments Fund, $16,890,941 in the Outpatient Directed Payments Fund, $15,761,554 in the Healthy DC Fund, and a net increase of $4,280,720 in DHCF's other four Dedicated Tax funds.

A proposed increase of $108,798 in Special Purpose Revenue (SPR) funds will align the personnel services budget across multiple divisions. This adjustment services includes a reduction of 0.6 FTE.

The Federal Grant funds proposed budget submission includes increases of $4,804,644 to align with projected grant awards across nonpersonnel services; and $468,804 and 3.0 FTEs to support salary and Fringe Benefit costs across multiple divisions.

The proposed budget submission also includes increases to Federal Medicaid Payments in the amount of $492,330,920 to account for the federal match for qualified services; and $1,424,204 and 6.4 FTEs in personnel service adjustments across multiple divisions.

Decrease: DHCF proposed Local funds also saved $361,882 in nonpersonnel services across multiple divisions, and a decrease of $6,222,832 in the DC Access System division. These savings result from ceasing to use recurring funds to support the costs related to equipment purchases.

In Dedicated Tax funds, DHCF proposed a decrease of $9,513 and 0.6 FTEs to align salaries and Fringe Benefit costs, and a decrease of $441,583 in the contractual services across multiple divisions to align with projected revenue.

The proposed budget submission for SPR funds includes a $1,625,242 decrease across multiple divisions will align the budget with projected revenues.

Enhance: DCHF's Local budget proposal will include a one-time increase of $6,531,248 across multiple divisions. This adjustment is comprised of $5,677,711 for software and equipment maintenance costs in the DCAS System division; $510,114 to support the upgrade and replacement of outdated computer equipment; $175,000 for a Memorandum of Understanding (MOU) with the Health Benefit Exchange Authority that supports In Vitro Fertilization treatment (IVF); and $168,423 in contractual services for professional development and administrative support. The Local funds budget proposal also includes a one-time increase of $6,509,686 in the Provider Services division to support the wage enhancement of Direct Support Professionals (DSP). The proposed Local funds budget also includes a net increase of $3,374,210 in subsidies in the Provider Services division to support the impacts of fewer beneficiaries enrolled in managed care. This adjustment is comprised of an increase $4,274,211 in Local spending to compensate for a loss in Dedicated Tax revenue, and is offset by a reduction of $900,001 to reflect savings in graduate medical education costs.

The proposed SPR budget includes an increase of $14,764 in nonpersonnel services across multiple divisions, which is comprised of $11,126 to the Agency Management division for contractual services; and $3,638 to the Provider Services division for subsidies.

The Federal Medicaid Payments funds proposed budget reflects an increase of $14,901,348 in subsidies to support match requirements for Local funded budget adjustments in the Provider Services division.

Reduce: The budget submission reflects a proposed one-time reduction of $137,717 in Local funds to step increases and associated fringe benefit costs across multiple divisions. The Local budget proposal also includes a net decrease of $11,363,413 in the Provider Services division to support provider rate reductions and limited enrollment for uninsured people otherwise not eligible for Medicaid.

The budget submission for Dedicated Taxes funds reflects a proposed one-time reduction of $7,385 to step increases and associated fringe benefit costs across multiple divisions. The Dedicated Taxes funds also includes a decrease of $34,260,077 in subsidies in the Provider Services division to align spending with lower projected revenue resulting, in part, from fewer beneficiaries enrolled in managed care services.

Lastly, the budget submission reflects proposed one-time reductions of $14,764 in Special Purpose Revenue funds; $9,803 in Federal Grant funds; and $141,499 in Federal Medicaid Payments to step increases and associated fringe benefit costs across multiple divisions.

District's Approved Budget

Enhance: DHCF's approved Local funds budget includes several increases in the Provider Services division. These adjustments include an increase of $2,297,155 that will be used to supplement lost inpatient revenue from updated Average Commercial Rate (ACR) projections. Additionally, a one-time Local fund increase of $1,801,051 is included to keep Alliance program enrollment open for those aged 21 to 25. An increase of $1,379,202 will be used to provide durable medical equipment for adults in the Alliance program; an increase of $475,000 in one-time funding reflects the additional cost of starting the Alliance enrollment closure for adults 26 and older two months later than originally planned and allowing those who enroll in FY 2026 and turn 26 in FY 2026 to remain enrolled in the Alliance program until September 30, 2026; and an increase of $345,287 is to support durable medical equipment coverage for youth under 26 years of age in the Alliance program.

Additionally in Local funds, an increase of $300,000 in the Health Care Reform and Innovation division is to support the Home Visiting/Nurse Family Partnership program; an increase of $197,678 in the Provider Services division is to support the continuation of all current methods of enrollment certification for the Alliance program; and lastly, an increase of $95,002 and 1.1 FTEs in the Agency Management division is to support the Health Care and Public Benefits Ombudsman program.

Reduce: In Local funds, DHCF's approved budget includes a reduction of $62,393 across multiple divisions resulting from cost savings for Information Technology (IT) hardware maintenance, training, membership dues, conference fees, travel and supplies. Also in Local funds, a reduction of $522,500 across multiple divisions is to reflect savings from the Realignment, Data Programming and Coding Support, and Behavioral Health Stakeholders Engagement contracts. Lastly, a reduction of $3,329,102 in the Provider Services division is to reflect the enrollment reforecast for the Alliance program.

FY 2026 Approved Full-Time Equivalents (FTEs)

Table HT0-7 contains the summary of FY 2026 Approved Budgeted Full-Time Equivalents (FTEs).

Table HT0-7

Total FY 2026 Approved Budgeted FTEs	**390.9**
Less: Interagency FTEs budgeted in this agency but employed by other agencies:	
BY0-Department of Aging and Community Living	(5.0)
CB0-Office of the Attorney General for the District of Columbia	(0.9)
HC0-Department of Health	(3.6)
HG0-Office of the Deputy Mayor for Health and Human Services	(1.0)
PO0-Office of Contracting and Procurement	(13.0)
TO0-Office of the Chief Technology Officer	(1.0)
Total Interagency FTEs budgeted in this agency, employed by other agencies	**(24.5)**
Add: Interagency FTEs budgeted in other agencies but employed by this agency:	
JA0-Department of Human Services	4.6
Total Interagency FTEs budgeted in other agencies, employed by this agency	**4.6**
Total FTEs employed by this agency	**371.0**

Note: Table HT0-7 displays the impact of the buyer agencies budgets funding the seller agencies FTEs in the FY 2026 budget, compared to how FTEs were budgeted in FY 2025.

-It starts with the FY 2026 budgeted FTE figure, 390.9 FTEs.

-It subtracts 24.5 FTEs budgeted in HT0 in FY 2026 who are employed by another agency.

-It adds 4.6 FTEs budgeted in other agencies in FY 2026 who are employed by HT0.

-It ends with 371.0 FTEs, the number of FTEs employed by HT0, which is the FTE figure comparable to the FY 2025 budget.